

IBUFEN SYRUP

MAL19984404AZ

DESCRIPTION:

SYRUP

Colour : Colourless
Flavour : Apple

EACH 5 ML OF SYRUP CONTAINS:

Ibuprofen 100 mg

Preservatives: Sodium Benzoate 0.1% w/v, Methyl Paraben 0.1% w/v & Propyl Paraben 0.01% w/v.

PHARMACODYNAMICS:

Analgesic action: It may block out pain impulse generation via a peripheral action which may involve inhibition of the synthesis of prostaglandins, and possibly inhibition of the synthesis or action of other substances which sensitise pain receptors to mechanical or chemical stimulation.

Anti-inflammatory action: It may act peripherally in inflamed tissue, probably by inhibiting the synthesis and/or actions of other local mediators of the inflammatory response.

Antipyretic action: Acts centrally on the hypothalamic heat-regulating centre to produce peripheral vasodilation, resulting in increased blood flow through the skin, sweating, and heat loss. The central action probably involves reduction of prostaglandin activity in the hypothalamus.

PHARMACOKINETICS:

Ibuprofen is absorbed from the gastrointestinal tract and peak plasma concentrations occur about 1 to 2 hours after ingestion. Ibuprofen is extensively bound to plasma proteins and has a half-life of about 2 hours. It is rapidly excreted in the urine mainly as metabolites and their conjugates. About 1% is excreted in urine as unchanged Ibuprofen and about 14% as conjugated Ibuprofen.

INDICATIONS:

It is indicated for fever and pain in children. Also for its analgesic and anti-inflammatory effects in the treatment of rheumatoid arthritis (including juvenile rheumatoid arthritis) and also soft tissue injuries such as sprains and strains. It is also indicated for the relief of mild to moderate pain such as dental and post-operative pain and for symptomatic relief of headache including migraine headache.

RECOMMENDED DOSE:

Children: 20 mg/kg body weight per day in divided doses. This can be achieved as follows:

1 - 2 years : One 2.5 ml spoonful (50 mg) three to four times daily.

3 - 7 years : One 5 ml spoonful (100 mg) three to four times daily.

8 - 12 years : Two 5 ml spoonfuls (200 mg) three to four times daily.

Not recommended for children weighing less than 7 kg or under the age of 1 year.

After assessing the risk/benefit ratio in each individual patient, the lowest effective dose for the shortest possible duration should be used.

ROUTE OF ADMINISTRATION:

FOR ORAL USE ONLY

CONTRAINDICATIONS:

Patients hypersensitive to Aspirin or Ibuprofen.

It should not be given to patients with active peptic ulceration.

WARNINGS AND PRECAUTIONS:

Ibuprofen should be given with care to patients with bleeding disorders, cardiovascular disease, and in those who are receiving coumarin anticoagulants. It should also be given with care to the elderly, to patients with asthma, bronchospasm and in liver or renal failure. Ibuprofen should be discontinued in patients who experience blurred or diminished vision, or changes in colour vision. Patients with collagen disease may be at risk of developing aseptic meningitis.

Cardiovascular Thrombotic Events

Observational studies have indicated that non-selective NSAIDs may be associated with an increased risk of serious cardiovascular events, principally myocardial infarction, which may increase with dose or duration of use. Patients with cardiovascular disease or cardiovascular risk of an adverse cardiovascular event in patient taking NSAID, especially in those with cardiovascular risk factors, the lowest effective dose should be used for the shortest possible duration.

There is no consistent evidence that the concurrent use of Aspirin mitigates the possible increased risk of serious cardiovascular thrombotic events associated with NSAID use.

Hypertension

NSAIDs may lead to the onset of new hypertension or worsening the pre-existing hypertension and patients taking antihypertensive with NSAIDs may have an impaired antihypertensive response. Caution is advised when prescribing NSAIDs to patients with hypertension. Blood pressure should be monitored closely during initiation of NSAID treatment and at regular intervals thereafter.

Heart Failure

Fluid retention and oedema have been observed in some patients taking NSAIDs, therefore caution is advised in patients with fluid retention or heart failure.

Gastrointestinal Events

All NSAIDs can cause gastrointestinal discomfort and rarely serious, potentially fatal gastrointestinal effects such as ulcers, bleeding and perforation which may increase with dose or duration of use, but can occur at any time without warning. Caution is advised in patients with risk factors for gastrointestinal events e.g. the elderly, those with a history of serious gastrointestinal events, smoking and alcoholism. When gastrointestinal bleeding or ulceration occurs in patients receiving NSAIDs, the drug should be withdrawn immediately. Doctors should warn patient about signs and symptoms of serious gastrointestinal toxicity. The concurrent use of Aspirin and NSAIDs also increases the risk of serious gastrointestinal adverse events.

Severe Skin Reactions

NSAIDs may very rarely cause serious cutaneous adverse events such as exfoliative dermatitis, toxic epidermal necrolysis (TEN) and Steven-Johnson Syndrome (SJS), which can be fatal and occur without warning. These serious adverse events are idiosyncratic and are independent of dose or duration of use. Patients should be advised of the signs and symptoms of serious skin reactions and to consult their doctor at the first appearance of a skin rash or any other sign of hypersensitivity.

WARNINGS

RISK OF GI ULCERATION, BLEEDING AND PERFORATION WITH NSAID

Serious GI toxicity such as bleeding, ulceration and perforation can occur at any time, with or without warning symptoms, in patients treated with NSAID therapy. Although minor GI problems (e.g. dyspepsia) are common, usually developing early in therapy, prescribers should remain alert for ulceration and bleeding in patients treated with NSAIDs even in the absence of previous GI tract symptoms.

Studies to date have not identified any subset of patients not at risk of developing peptic ulceration and bleeding. Patients with prior history of serious GI events and other risk factors associated with peptic ulcer disease (e.g. alcoholism, smoking, corticosteroid therapy) are at increased risk. Elderly or debilitated patients seem to tolerate ulceration or bleeding less than other individuals and account for most spontaneous reports for fatal GI events.

DRUG INTERACTIONS:

Oral anticoagulants and thiazide diuretics.

PREGNANCY AND LACTATION:

Whilst no teratogenic effects have been demonstrated in animal toxicology studies, the use of ibuprofen during pregnancy should, if possible, be avoided. Congenital abnormalities have been reported in association with ibuprofen administration in man; however, these are low in frequency and do not appear to follow any discernible pattern. In view of the known effects of NSAIDs on the foetal cardiovascular system (closure of ductus arteriosus), use in late pregnancy should be avoided.

In the limited studies so far available, ibuprofen appears in the breast milk in very low concentrations and is unlikely to adversely affect the breast-fed infant.

SIDE EFFECTS:

Gastrointestinal disturbances, peptic ulceration, gastrointestinal bleeding, headache, dizziness, nervousness, skin rash, pruritus, tinnitus, oedema, depression, drowsiness, insomnia, blurred vision, cystitis, haematuria, interstitial nephritis, nephrotic syndrome, anaemias, neutropenia, eosinophilia, toxic amblyopia and other ocular reactions. Hypersensitivity reactions, abnormalities of liver functions tests, impairment of renal function, agranulocytosis and thrombocytopenia have occasionally been observed.

SYMPTOMS AND TREATMENT OF OVERDOSE:

Symptoms include vomiting and gastrointestinal haemorrhage.

Treatment is by aspiration and lavage. Vital functions should be maintained.

PACKINGS/PACK SIZE(S):

Plastic bottles of 60 ml and 100 ml.

**JAUHI UBAT DARIPADA KANAK-KANAK
KEEP OUT OF REACH OF CHILDREN
SHAKE WELL BEFORE USE**

MANUFACTURER/PRODUCT REGISTRATION HOLDER:

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